

D121

Health Promotion of Patients and Populations Across the Lifespan

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A county health department that recently won a state innovation award runs a free mammography van for women ages 50 to 74 who have no breast symptoms. The van visits rural churches on Saturdays and is staffed by two technologists. What level of prevention does this program represent?

- A. Secondary prevention.
- B. Primary prevention.
- C. Primary prevention, because it is delivered in the community.
- D. Tertiary prevention.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Which activity is an example of tertiary prevention?

- A. Blood pressure screening at a health fair.
- B. Influenza vaccination for adults.
- C. Any service delivered in a hospital.
- D. Cardiac rehabilitation after a heart attack.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A 45-year-old accountant with no symptoms asks for a screening test that the USPSTF grades D for adults at average risk. She read about it in a popular wellness magazine and her employer's insurance would cover it. What is the most appropriate response?

- A. Explain that the evidence is insufficient to decide.
- B. Offer it selectively based on her preferences.
- C. Order it, since her insurance will cover it.
- D. Explain that the evidence shows no net benefit or that harms outweigh benefits, and discourage the test.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A nonprofit plans to screen 10,000 adults at a cost of \$25 per test. The condition has a prevalence of 2% in this group. The test has 90% sensitivity and 95% specificity. Of the people who test positive, about what percentage actually have the condition?

- A. About 27%
- B. 90%
- C. 2%
- D. About 37%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

5. [TRANSFER — reversed stem / different variable]

The same test (90% sensitivity, 95% specificity) is used in a high-risk clinic where prevalence is 20%. What is the positive predictive value now?

- A. About 27%, because the test has not changed.
- B. About 18%
- C. About 82%
- D. 90%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

6.

In a two-year trial run at 14 sites, 10% of adults in the usual-care group had a stroke compared with 6% in the group receiving a new outreach program. The program was designed with input from community leaders. How many people must receive the program to prevent one stroke?

- A. 4
- B. 25
- C. 17
- D. 2.5

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

7. [TRANSFER — reversed stem / different variable]

A program has an NNT of 50. Without it, 5% of people have the outcome. What percentage have the outcome with the program?

- A.** 7%
- B.** 2%
- C.** 3%
- D.** 4.9%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A nurse practitioner at a clinic with a new electronic health record notices that many patients with asthma live in older apartment buildings with mold and pests, near a highway. The clinic manager just finished an MBA. Which SDOH domain best describes this factor?

- A.** Neighborhood and built environment.
- B.** Economic stability.
- C.** Individual health behavior.
- D.** Health care access and quality.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A 52-year-old bus driver knows colon cancer runs in his family and believes screening could save his life. He has seen three TV ads about screening this month. He says he cannot take a day off for a colonoscopy without losing pay. Using the Health Belief Model, what is the best intervention?

- A.** Add more reminder ads as cues to action.
- B.** Offer a stool-based home test to remove the time-off barrier.
- C.** Tell him screening is required for everyone his age regardless of cost.
- D.** Give him more education on how deadly colon cancer is.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A 40-year-old who has smoked for 20 years and works night shifts at a warehouse says, “I know I should quit, and I probably will sometime in the next few months, but I’m not ready to set a date.” His wife quit last year. Which stage of change is he in?

- A. Preparation.
- B. Precontemplation.
- C. Action, because his wife already quit.
- D. Contemplation.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

11. [TRANSFER — reversed stem / different variable]

A patient is in the preparation stage for increasing physical activity. Which statement best fits that stage?

- A. “I’ve walked daily for two months now.”
- B. “I’ve been walking every day for a year.”
- C. “I joined a gym yesterday and will start classes next week.”
- D. “Maybe someday, but exercise isn’t for me.”

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

12.

A health system with a large rural footprint must choose one diabetes program. Program A costs \$300,000 and yields 20 QALYs. Program B costs \$500,000 and yields 30 QALYs. Program B would be hosted by a local church with 400 members. What is the incremental cost-effectiveness ratio of B compared with A?

- A. \$500 per church member
- B. \$20,000 per QALY
- C. About \$16,667 per QALY
- D. \$15,000 per QALY

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer A Concept 1

A county health department ~~that recently won a state innovation award~~ runs a free mammography van for women ages 50 to 74 who have no breast symptoms. ~~The van visits rural churches on Saturdays and is staffed by two technologists.~~ What level of prevention does this program represent?

Target word: level of prevention **Decides:** Screening people without symptoms to find existing disease early.

A	Correct	Screening finds disease early in people without symptoms.
B	Adjacent category	Primary prevention stops disease from starting; mammography does not prevent cancer.
C	Camouflage	The setting does not decide the level; the disease stage does.
D	Confusion set	Tertiary prevention reduces harm from disease already diagnosed.

2. Answer D Concept 1 · transfer

Which activity is an example of tertiary prevention?

Target word: reverse: pick an activity for a given level **Decides:** Tertiary = limiting harm from diagnosed disease.

A	Adjacent category	Screening is secondary prevention.
B	Inverted operation	Vaccination is primary prevention.
C	Absolute language	Level depends on disease stage, not setting.
D	Correct	It limits disability and recurrence in diagnosed disease.

3. Answer D Concept 2

A 45-year-old accountant with no symptoms asks for a screening test that the USPSTF grades D for adults at average risk. ~~She read about it in a popular wellness magazine and her employer's insurance would cover it.~~ What is the most appropriate response?

Target word: meaning of grade D **Decides:** Grade D = recommend against; harms outweigh benefits or no net benefit.

A	Confusion set	That describes grade I, not D.
B	Adjacent category	That describes grade C.
C	Camouflage	Coverage does not change the evidence grade.
D	Correct	Grade D means the USPSTF recommends against offering the service.

4. Answer A Concept 3

A nonprofit plans to screen 10,000 adults at a cost of \$25 per test. The condition has a prevalence of 2% in this group. The test has 90% sensitivity and 95% specificity. Of the people who test positive, about what percentage actually have the condition?

Target word: positive predictive value **Decides:** True positives = 180; false positives = 490.

A	Correct	200 have it $\times 0.90 = 180$ true positives; $9,800 \times 0.05 = 490$ false positives; $180 / 670 \approx 27\%$.
B	Confusion set	Sensitivity is the share of sick people who test positive, not the share of positives who are sick.
C	Wrong stem number	The prevalence, not the predictive value.
D	Inverted operation	Divides true positives by false positives ($180 / 490$) instead of by all positives.

5. Answer C Concept 3 • transfer

The same test (90% sensitivity, 95% specificity) is used in a high-risk clinic where prevalence is 20%. What is the positive predictive value now?

Target word: PPV at a new prevalence **Decides:** Per 1,000: 180 true positives, 40 false positives.

A	Dropped element	PPV depends on prevalence, not just the test.
B	Inverted operation	That is the share of positives that are false ($40 / 220$).
C	Correct	$200 \times 0.90 = 180$; $800 \times 0.05 = 40$; $180 / 220 \approx 82\%$.
D	Confusion set	That is sensitivity, which does not change with prevalence.

6. Answer B Concept 4

In a two-year trial run at 14 sites, 10% of adults in the usual-care group had a stroke compared with 6% in the group receiving a new outreach program. ~~The program was designed with input from community leaders.~~ How many people must receive the program to prevent one stroke?

Target word: NNT **Decides:** $ARR = 10\% - 6\% = 4\%$.

A	Dropped element	Stops at the ARR (4 percentage points) without inverting it.
B	Correct	$1 / 0.04 = 25$.
C	Wrong stem number	$1 / 0.06$ uses the treatment group's rate alone.
D	Confusion set	Uses the relative risk reduction (40%), not the absolute reduction.

7. Answer C Concept 4 • transfer

A program has an NNT of 50. Without it, 5% of people have the outcome. What percentage have the outcome with the program?

Target word: solve for the treatment event rate **Decides:** $ARR = 1 / 50 = 2\%$.

A	Inverted operation	Adds the reduction instead of subtracting it.
B	Dropped element	That is the ARR, not the remaining event rate.
C	Correct	$ARR = 2\%$; $5\% - 2\% = 3\%$.
D	Confusion set	Treats 2% as a relative reduction of 5%.

8. Answer A Concept 5

A nurse practitioner ~~at a clinic with a new electronic health record~~ notices that many patients with asthma live in older apartment buildings with mold and pests, near a highway. ~~The clinic manager just finished an MBA.~~ Which SDOH domain best describes this factor?

Target word: SDOH domain **Decides:** Housing quality and nearby pollution are the physical environment.

A	Correct	Housing conditions and environmental exposures belong to this domain.
B	Adjacent category	Income may explain where people live, but the exposure itself is environmental.
C	Confusion set	Behavior is not an SDOH domain; this is a condition where people live.
D	Camouflage	Answers the clinic and EHR details, which do not describe the exposure.

9. Answer B Concept 6

A 52-year-old bus driver knows colon cancer runs in his family and believes screening could save his life. ~~He has seen three TV ads about screening this month.~~ He says he cannot take a day off for a colonoscopy without losing pay. Using the Health Belief Model, what is the best intervention?

Target word: which HBM construct to address **Decides:** Susceptibility and benefits are already high; the barrier is time off.

A	Camouflage	He already has cues; more ads do not remove the barrier.
B	Correct	Reducing the perceived barrier is the highest-yield step when other beliefs are already in place.
C	Absolute language	Pressure does not address the barrier and misstates shared decision-making.
D	Adjacent category	Targets perceived severity, which is not the obstacle.

10. Answer D Concept 7

A 40-year-old who has smoked for 20 years and ~~works night shifts at a warehouse~~ says, "I know I should quit, and I probably will sometime in the next few months, but I'm not ready to set a date." ~~His wife quit last year.~~ Which stage of change is he in?

Target word: stage of change **Decides:** Intends to change within 6 months but no date or steps.

A	Adjacent category	Preparation means planning within 30 days with steps taken.
B	Inverted operation	Precontemplation means no intent to change; he states intent.
C	Camouflage	His wife's quitting does not decide his stage.
D	Correct	He intends to change within about 6 months but is not ready to plan.

11. Answer C Concept 7 · transfer

A patient is in the preparation stage for increasing physical activity. Which statement best fits that stage?

Target word: reverse: statement from stage **Decides:** Preparation = plan within 30 days, small steps taken.

A	Adjacent category	Changed behavior under 6 months is action.
B	Inverted operation	Over 6 months of change is maintenance.
C	Correct	Concrete near-term plan with a step already taken.
D	Confusion set	No intent is precontemplation.

12. Answer B Concept 8

A health system ~~with a large rural footprint~~ must choose one diabetes program. Program A costs \$300,000 and yields 20 QALYs. Program B costs \$500,000 and yields 30 QALYs. ~~Program B would be hosted by a local church with 400 members.~~ What is the incremental cost-effectiveness ratio of B compared with A?

Target word: ICER **Decides:** Extra cost \$200,000 / extra 10 QALYs.

A	Camouflage	Uses the membership count, which does not measure health gained.
B	Correct	$(\$500,000 - \$300,000) / (30 - 20) = \$20,000$ per QALY gained.
C	Confusion set	B's average ratio ($\$500,000 / 30$), not the increment over A.
D	Wrong stem number	A's average ratio ($\$300,000 / 20$).

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	3	
Wrong stem number	A figure copied from the prompt that is not the answer.	3	
Adjacent category	The right idea in the wrong bucket.	7	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	6	
Absolute language	Always, never, guarantee — where judgment applies.	2	
Confusion set	Every option is real; the test is exact role-matching.	9	
Camouflage	Answers a stem fact that was never the question.	6	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Levels of prevention	Primary prevention stops disease before it starts (vaccines, counseling); secondary finds it early in people without symptoms (screening); tertiary limits harm from disease already present (rehab, complication prevention).	Calling any screening test primary prevention, or any activity for a sick person secondary.	Whether the person already has the disease, and whether it is known.
2	USPSTF recommendation grades	Grades A and B mean offer the service; C means offer selectively based on judgment and preference; D means recommend against; I means evidence is insufficient to weigh benefits and harms.	Reading grade D or I as “optional, do it if the patient wants.”	A letter grade attached to a screening or preventive service.
3	Screening test accuracy and predictive value	Sensitivity and specificity belong to the test; positive predictive value depends on prevalence, so the same test yields many more false positives in a low-risk population.	Treating sensitivity as the chance a positive result is true.	“Of those who test positive, what share actually have the disease?”
4	Absolute risk reduction and number needed to treat	ARR = control event rate – treatment event rate; NNT = 1 / ARR.	Using the relative risk reduction to calculate NNT.	Two event rates for intervention and comparison groups.
5	Social determinants of health domains	Healthy People 2030 groups SDOH into five domains: economic stability, education access and quality, health care access and quality, neighborhood and built environment, and social and community context.	Treating individual behaviors or genetics as social determinants.	A condition where people live, learn, work, or play that shapes health.
6	Health Belief Model	Action depends on perceived susceptibility, perceived severity, perceived benefits, perceived barriers, cues to action, and self-efficacy; lowering the strongest barrier is often the highest-yield move.	Adding more education on severity when the real problem is a barrier.	A person who believes the risk but names a practical obstacle.

#	Concept	Rule	Trap	Tell
7	Stages of change (Transtheoretical Model)	Precontemplation (no intent within 6 months), contemplation (intends within 6 months), preparation (within 30 days, with steps taken), action (under 6 months of change), maintenance (over 6 months); match the intervention to the stage.	Giving an action plan to someone who is still ambivalent.	A time frame or statement of intent about changing a behavior.
8	Cost-effectiveness with finite resources	Compare programs with the incremental cost-effectiveness ratio: extra cost divided by extra health gained (e.g., per QALY) relative to the alternative.	Comparing each program's average cost per QALY instead of the increment.	Two programs, each with a cost and an outcome total.

Course scope (public description): Health Promotion of Patients and Populations Across the Lifespan prepares the graduate nursing student to perform the role of an advanced practice nurse with the essential competencies and skills necessary to provide health promotion activities to individuals and populations. This course will prepare the graduate nursing student to incorporate individual characteristics, population factors, and social determinants of health (SDOH) in determining the most efficient use of finite resources in leading health promotion activities. This course will also prepare the graduate nursing student to lead health promotion activities for individuals and specific populations across the lifespan. All MSN Core courses, as well as Advanced Pathophysiology for the Advanced Practice Nurse, Advanced Pharmacology for the Advanced Practice Nurse, and Advanced Health Assessment for the Advanced Practice Nurse must be completed before taking this course.